[redacted] False, indisputable

[redacted] False, because Gie City lied/withheld information

[redacted] False/inaccurate, disputable

[redacted] Subjective - projecting their values onto Gie City's life

Government of South Australia
SA Health, Operations
Adelaide Metro Mental Health Directorate
Eastern Community Mental Health

Office Address
Eastern Community Mental Health Centre
172 Glynburn Road
Tranmere SA 5073

Postal Address
PO Box 4097
Tranmere North SA 5079

Tel: (08) 7425 5555
Fax: (08)7425 5556

25 FEB 2016

DATE: 24/02/16

Dear Dr Wong,
Tusmore Surgery,
103 Tussmore Ave, SA

RE: Giles Cullity DoB: 22/07/1993

Diagnosis: Schizoaffective disorder

The client did attend his scheduled appointment today at Tranmere Community Mental Health Centre.

From this appointment the relevant clinical information / mental state examination details are:

Mr Giles Cullity is a 22 year old single male, currently living with his parents and 3 siblings in a family home in Leabrook. He was previously on home detention bail with monitoring device on ankle for attempted murder charge; matter has since finalised with Giles receiving 3 year suspended sentence. Giles has been diagnosed with BPAD (DDX Schizoaffective disorder) and Aspergers syndrome traits following index admission May 2013 (Flinders 5J, transferred to RAH C3). He is currently on following medications:
Risperidone 2mg nocte, po;
Citalopram 20mg daily, po.

I ~~further~~ gather that he was on Depot Paliperidone till recently which was ceased and he is currently on oral Risperidone.

During the review today, Giles presented in calm, polite and respectful manner. He reported doing well these days. He claimed his attention, concentration and ability to work on his dream project (to invent a search engine on internet) have improved since depot medication was ceased. He argued against the depot medication and believed it being cause of his low motivation. He however was willing to continue taking Risperidone and Citalopram in above doses. He denied any major adverse effects related to above medications.

Giles stated his sleep and appetite have been normal. He spends most of his time at home in front of the computer. He also talked about doing exercise to keep himself fit.

Giles believed that he has endocrinological problem which makes his physical appearance look childlike. He believes that his testosterone receptors are hypoactive and hence the muscle mass is small and his shoulder are less broad for his age. He then talked about nervous and musculoskeletal problems. He claimed that his muscles do not contract properly. He however

did not appear pre-occupied with above beliefs nor sought any investigations in to above issues.

Giles denied any other psychotic experiences including paranoia, referential beliefs, hallucinations, thought alienation and somatic passivity phenomena. He reported his mood to be stable these days and denied any depressive cognition, death wishes, and suicidal ideations. He also denied any aggressive thoughts to hurt others.

Giles denied using any illicit drugs or alcohol these days. As part of his suspended statement, he meets the corrections officer every 4 weeks.

Giles sought information about Cyclothymia and believed that he may have that disorder instead of schizoaffective disorder or BPAD. I explained to him about above disorder and also gave a brief summary of medications used to treat above disorder upon his request.

MSE: Giles appeared moderately built, nourished Caucasian male of stated age. He was well kempt, alert and oriented. He avoided direct eye contact however rapport could be established. He spoke relevantly and coherently with no evidence of formal thought disorder. He expressed ongoing somatic beliefs which appeared to be overvalued ideas (likely of delusional quality). The beliefs included having an endocrinological problems leading to his poor muscle mass and younger looks an stated age. He further had odd and eccentric beliefs around his ability to create new computer program. He denied any other psychotic experiences, death wishes or depressive cognitions. His affect appeared blunted, restricted with limited reactivity. He has partial insight and his current judgment appeared intact.

O/E: Weight: 89.9kg; BP: 128/66mmHg; Pulse: 62/min; No EPSE noted.

Plan:
Continue Risperidone and Citalopram in current doses.
I would suggest for psychological interventions for body dysmorphia symptoms however he has poor insight in to it.
Follow up in Tranmere medical clinic after 6-8 weeks.

If you wish to discuss any of the above or other issues please contact me as above.

Yours sincerely

Dr Sandip Kulkarni,
Psychiatry Registrar,
Eastern Community Mental Health Service, Tranmere 5073.